Colon Cancer AI Concierge Safety Protocol

Emergency & Urgent Symptom Recognition Guide

For Colon Cancer Patients During and After Treatment

IMPORTANT: This guide helps identify when to seek care. When in doubt, always err on the side of caution and contact your medical team or seek emergency care.

SECTION 1: EMERGENCY SYMPTOMS

CALL 911 OR GO TO EMERGENCY ROOM IMMEDIATELY

These symptoms indicate potentially life-threatening conditions requiring immediate emergency medical intervention. Do not wait—call 911 or have someone drive you to the nearest emergency room.

1.1 CARDIOVASCULAR EMERGENCIES

Can occur with bevacizumab (Avastin), 5-FU, capecitabine, or underlying disease

- **Chest pain or pressure** — May indicate heart attack, especially with 5-FU or capecitabine
- **Severe shortness of breath at rest** — Could indicate pulmonary embolism, heart failure, or severe anemia
- **Sudden severe leg pain with swelling** — Signs of deep vein thrombosis (DVT) which can lead to pulmonary embolism
- **Sudden difficulty breathing with chest pain** — Classic pulmonary embolism presentation—cancer patients at higher risk
- **Racing heart with dizziness or fainting** — May indicate arrhythmia or severe dehydration
- **Coughing up blood** — Could indicate pulmonary embolism or severe bleeding

1.2 NEUROLOGICAL EMERGENCIES

Can occur with brain metastases, severe electrolyte imbalances, or stroke

- **Sudden severe headache ('worst headache of life')** — May indicate brain bleed or stroke
- **Sudden confusion or altered mental status** — Could indicate brain metastases, stroke, severe infection, or metabolic crisis
- **Sudden weakness on one side of body** — Classic stroke symptom—time critical
- **Sudden difficulty speaking or understanding speech** — Stroke warning sign
- **Sudden vision changes or loss** — Could indicate stroke or increased intracranial pressure
- **Seizures (new onset)** — May indicate brain involvement, severe electrolyte imbalance, or medication effect
- **Loss of consciousness** — Multiple potential causes requiring immediate evaluation

1.3 BLEEDING EMERGENCIES

Higher risk with bevacizumab, low platelets from chemo, or anticoagulants

- **Vomiting blood or material that looks like coffee grounds** — Upper GI bleeding—potentially life-threatening
- **Large amounts of bright red blood from rectum** — May indicate tumor bleeding, severe hemorrhage
- **Black, tarry stools (melena) with dizziness** — Indicates significant upper GI bleeding
- **Uncontrolled bleeding that won't stop** — Risk with low platelets or anticoagulation
- **Signs of internal bleeding** — Severe abdominal pain with rigid abdomen, rapid pulse, pale/clammy skin

1.4 SEPSIS / SEVERE INFECTION

Chemotherapy suppresses immune system—infections can rapidly become life-threatening

- **Fever ≥100.4°F (38°C) with shaking chills during chemotherapy** — Febrile neutropenia is a medical emergency
- **Fever with rapid breathing, rapid heart rate, and confusion** — Signs of sepsis—life-threatening infection
- **Fever with low blood pressure or dizziness upon standing** — Septic shock warning signs
- **Fever that won't respond to acetaminophen** — May indicate serious infection

1.5 GASTROINTESTINAL EMERGENCIES

Related to tumor, surgery, or treatment complications

- **Severe abdominal pain with rigid, board-like abdomen** — May indicate bowel perforation—surgical emergency
- **Complete inability to pass gas or stool with vomiting** — Signs of complete bowel obstruction
- **Severe abdominal pain with fever and distension** — Could indicate perforation, abscess, or severe infection
- **Fecal vomiting (vomit that smells like stool)** — Late sign of complete bowel obstruction

1.6 POST-SURGICAL EMERGENCIES

Within weeks of colon surgery, colostomy creation, or port placement

- **Wound opening with visible internal tissue** — Wound dehiscence requiring immediate attention
- **Sudden severe pain at surgical site with fever** — May indicate anastomotic leak or abscess

- **Ostomy turning dark purple, black, or gray** — Stoma necrosis—blood supply compromised
- **No output from ostomy for >6 hours with nausea/distension** — Possible obstruction
- **Port site with spreading redness, warmth, and fever** — Port infection potentially entering bloodstream

1.7 SEVERE ALLERGIC REACTIONS

Can occur with any infused medication, especially cetuximab, oxaliplatin

- **Difficulty breathing or swallowing during/after infusion** — Anaphylaxis—life-threatening allergic reaction
- **Swelling of face, lips, tongue, or throat** — Angioedema—airway emergency
- **Widespread hives with dizziness or fainting** — Severe systemic allergic reaction
- **Feeling of 'impending doom' with any above symptoms** — Classic anaphylaxis symptom

1.8 IMMUNOTHERAPY-RELATED EMERGENCIES

For patients on pembrolizumab, nivolumab, ipilimumab, dostarlimab

- **Severe shortness of breath or new oxygen need** — May indicate immune-related pneumonitis
- **Severe abdominal pain with bloody diarrhea (>6 episodes/day)** — Severe immune-related colitis
- **Yellowing of skin/eyes with dark urine** — Immune-related hepatitis
- **Severe headache with vision changes and neck stiffness** — Possible immune-related encephalitis or meningitis
- **Extreme muscle weakness, difficulty breathing** — Could indicate myocarditis or myasthenia-like syndrome
- **Severe skin blistering or peeling** — Stevens-Johnson syndrome or toxic epidermal necrolysis

SECTION 2: URGENT SYMPTOMS

CONTACT YOUR ONCOLOGIST OR CANCER CENTER SAME DAY

These symptoms require prompt medical evaluation—contact your oncology team the same day. If after hours, call the on-call oncologist or nurse line. Do not wait until your next scheduled appointment.

2.1 FEVER & INFECTION SIGNS

Immunocompromised patients can deteriorate quickly

- **Fever 100.4°F (38°C) or higher** — During chemo, any fever needs evaluation—do not take Tylenol and wait
- **Temperature below 96°F (35.5°C)** — Hypothermia can also indicate serious infection
- **Chills or rigors without measured fever** — May indicate bacteremia even without documented fever
- **Productive cough with colored sputum** — Possible pneumonia in immunocompromised patient
- **Burning with urination or cloudy/bloody urine** — Urinary tract infection that can quickly progress
- **New mouth sores with difficulty eating/drinking** — Mucositis with infection risk, dehydration concern
- **Redness, warmth, swelling around port or IV site** — Early signs of line infection
- **New skin wound or cut that looks infected** — Impaired healing and infection risk during treatment

2.2 GASTROINTESTINAL SYMPTOMS

Common with chemo but can become serious quickly

- **Diarrhea: >4 episodes in 24 hours** — Risk of severe dehydration and electrolyte imbalance
- **Diarrhea with blood or mucus** — May indicate colitis or infection (C. diff)
- **Diarrhea that doesn't respond to loperamide after 24 hours** — May need IV fluids and further workup
- **Vomiting: Unable to keep down fluids for 12+ hours** — Dehydration risk, may need IV fluids and antiemetics
- **Vomiting: >4-6 episodes in 24 hours despite anti-nausea meds** — Antiemetic regimen may need adjustment
- **New or worsening abdominal pain** — Could indicate obstruction, infection, or other complication

- **Abdominal distension with decreased bowel sounds** — Possible partial obstruction or ileus
- **No bowel movement for 3+ days with discomfort** — Constipation can progress to obstruction
- **New rectal bleeding (more than streaks on toilet paper)** — Needs evaluation to determine source

2.3 DEHYDRATION WARNING SIGNS

Common complication of diarrhea, vomiting, poor intake

- **Dizziness or lightheadedness when standing** — Orthostatic hypotension from volume depletion
- **Dark yellow or amber urine** — Concentrated urine indicates dehydration
- **Urinating much less than usual** — Decreased output indicates fluid deficit
- **Dry mouth and extreme thirst** — Body signaling fluid needs
- **Rapid heart rate at rest (>100 bpm)** — Compensatory response to low volume
- **Muscle cramps** — Often from electrolyte imbalances with dehydration

2.4 PAIN SYMPTOMS

New or changing pain patterns need evaluation

- **New bone pain not explained by activity** — Could indicate bone metastases
- **Severe headache different from usual pattern** — Needs evaluation, especially if worsening
- **Back pain with leg weakness or numbness** — Could indicate spinal cord compression—urgent!
- **Pain not controlled by current medications** — May need dose adjustment or workup for cause
- **Chest pain (mild) without emergency features** — Still needs same-day evaluation
- **Severe mouth/throat pain preventing eating** — Mucositis needs management

2.5 BREATHING SYMPTOMS

Can indicate infection, blood clot, fluid, or drug reaction

- **New shortness of breath with activity** — Could indicate anemia, infection, or pulmonary issue
- **New or worsening cough** — Needs evaluation for infection or other causes
- **Cough that produces blood-tinged sputum** — Needs prompt investigation

- **Wheezing or noisy breathing (new)** — Could indicate allergic reaction or infection
- **Needing extra pillows to breathe at night** — Sign of fluid accumulation

2.6 NEUROLOGICAL SYMPTOMS

May be drug-related or indicate disease progression

- **New numbness or tingling in hands/feet** — Neuropathy—may need dose adjustment
- **Difficulty with fine motor tasks (buttons, writing)** — Worsening neuropathy
- **New difficulty walking or balance problems** — Could indicate neuropathy or other issue
- **Jaw pain or tightness triggered by cold (with oxaliplatin)** — Acute neuropathy—need to avoid cold
- **Throat tightness with swallowing cold items** — Oxaliplatin laryngopharyngeal dysesthesia
- **Memory changes or 'chemo brain' that's worsening** — May need evaluation
- **Ringing in ears (tinnitus)** — Can be drug-related, needs documentation
- **Hearing changes** — Some drugs can affect hearing

2.7 SKIN & APPEARANCE CHANGES

Can indicate drug reactions, infection, or other complications

- **New rash covering large body area** — Could be drug reaction or infection
- **Rash with blisters or peeling skin** — Severe reaction needs immediate attention
- **Acne-like rash with EGFR inhibitors (severe)** — May need dose modification or dermatology
- **Hand-foot syndrome with blistering or severe pain** — Capecitabine/5-FU side effect needing management
- **Swelling in legs, ankles, or around eyes (new)** — Could indicate fluid retention or kidney issue
- **Yellowing of skin or eyes** — Jaundice—needs urgent liver evaluation
- **Unusual bruising or petechiae (tiny red spots)** — May indicate low platelets

2.8 BLOOD PRESSURE CONCERNS

Particularly important with bevacizumab (Avastin)

- **Blood pressure >180/110 mmHg** — Severe hypertension needs treatment
- **Severe headache with high blood pressure** — Hypertensive urgency

- **New or significantly worsening hypertension** — Common with anti-VEGF drugs, needs management
- **Nosebleeds that are difficult to stop** — Can occur with hypertension or low platelets

2.9 OSTOMY-SPECIFIC URGENT SYMPTOMS

For patients with colostomy or ileostomy

- **Stoma color change to pale, dusky, or dark red** — Circulation concern
- **Stoma pulled below skin level (retraction)** — May cause leakage and skin damage
- **Stoma protruding much more than usual (prolapse)** — Needs evaluation
- **Skin around stoma severely irritated/broken down** — Infection risk, needs wound care
- **Significantly increased or decreased output** — Could indicate dehydration or obstruction
- **Watery ileostomy output >1500mL per day** — High output, dehydration risk
- **Blood in ostomy output (more than occasional)** — Needs investigation

2.10 IMMUNOTHERAPY-SPECIFIC URGENT SYMPTOMS

Immune-related adverse events can progress quickly

- **Diarrhea: 4-6 watery stools above baseline** — Early immune colitis—steroids may be needed
- **New or worsening fatigue with cold intolerance** — Possible thyroid dysfunction
- **Increased thirst, frequent urination, weight loss** — Could indicate immune-related diabetes
- **Joint pain affecting multiple joints** — Immune-related arthritis
- **New skin rash or itching (moderate)** — Immune-related dermatitis
- **Dry eyes, dry mouth (new, significant)** — Could indicate immune-related syndrome

SECTION 3: NEEDS ATTENTION WITHIN 24-48 HOURS

REPORT TO YOUR CARE TEAM AT NEXT OPPORTUNITY

These symptoms should be reported to your oncology team within 24-48 hours or at your next scheduled contact. Keep notes to share with your provider.

3.1 DIGESTIVE SYMPTOMS (MILD-MODERATE)

- **Mild nausea controlled by anti-nausea medications** — Track pattern, may need regimen adjustment
- **Appetite changes or food aversions** — Common, but document for nutritional support
- **Mild diarrhea (2-3 loose stools above normal)** — Monitor, increase fluids, report if worsens
- **Constipation responding to stool softeners** — Continue management, report if worsens
- **Heartburn or indigestion (new or worsening)** — May need medication adjustment
- **Taste changes (metallic taste, food tastes different)** — Very common, affects nutrition
- **Mild mouth sores not affecting eating** — Needs documentation, may worsen
- **Bloating or gas (increased)** — Common, track pattern

3.2 FATIGUE & ENERGY CHANGES

- **Fatigue interfering with daily activities** — Very common, but severity should be tracked
- **Sleep disturbances (insomnia, frequent waking)** — Can worsen other symptoms
- **Excessive sleeping or drowsiness** — Track duration and impact
- **'Chemo brain' (mild cognitive fog)** — Common, document specific issues
- **Decreased motivation or interest** — Could indicate depression, needs attention
- **Weakness in muscles (mild, general)** — Common during treatment

3.3 SKIN, HAIR & NAIL CHANGES

- **Hair thinning or loss** — Emotionally significant, discuss options
- **Skin dryness or itching (mild)** — Needs moisture management
- **Mild rash or skin color changes** — Document location and appearance

- **Nail changes (ridges, discoloration, brittleness)** — Common with some regimens
- **Increased sun sensitivity** — Common, need sun protection guidance
- **Mild hand-foot syndrome (redness, tingling)** — May progress, needs monitoring
- **Acne-like rash (mild) with EGFR inhibitors** — Actually correlates with response, but needs management

3.4 NEUROPATHY (MILD)

- **Mild cold sensitivity (with oxaliplatin)** — Expected, avoid cold foods/drinks/air
- **Occasional tingling that resolves** — Early neuropathy, needs documentation
- **Slight difficulty with buttons or small objects** — Early fine motor impact
- **Mild numbness in fingertips or toes** — Document which fingers/toes, when it occurs

3.5 EMOTIONAL & PSYCHOLOGICAL SYMPTOMS

- **Persistent sadness or tearfulness** — May need mental health support
- **Anxiety about treatment or disease** — Very common, support available
- **Mood swings or irritability** — Can be hormone or medication related
- **Withdrawal from family or friends** — Concerning pattern to address
- **Feeling overwhelmed by diagnosis or treatment** — Normal, but support helps
- **Loss of interest in activities previously enjoyed** — Screen for depression

3.6 OTHER SYMPTOMS TO TRACK

- **Weight changes (loss or gain >5 lbs in a week)** — Needs nutritional evaluation
- **Mild swelling in ankles/feet (end of day)** — Track, may worsen
- **Hot flashes or sweating** — Can occur with some treatments
- **Muscle aches or joint stiffness** — Common, may need management
- **Eye irritation or watering** — Can occur with some drugs
- **Changes in menstrual periods** — Treatment effect on hormones
- **Sexual function changes** — Important quality of life issue
- **Mild nosebleeds** — Track frequency

SECTION 4: QUICK REFERENCE SUMMARY

Use this summary table for rapid decision-making:

CALL 911 — EMERGENCY
• Chest pain or difficulty breathing • Stroke symptoms (sudden weakness, speech difficulty, vision changes)
• Uncontrolled bleeding • Fever with shaking chills during chemo • Severe allergic reaction
• Severe abdominal pain with rigid belly • Loss of consciousness • Seizures
• Signs of sepsis (fever + rapid breathing + confusion)

CALL ONCOLOGIST SAME DAY — URGENT
• Fever ≥100.4°F (38°C) • Diarrhea >4 episodes/day • Unable to keep fluids down for 12+ hours
• Signs of dehydration (dizziness, dark urine, rapid pulse) • Blood in stool (new)
• New numbness/tingling • Back pain with leg weakness • Pain not controlled by meds
• Spreading rash or skin blistering • Yellowing of skin/eyes • Ostomy color changes

REPORT WITHIN 24-48 HOURS — NEEDS ATTENTION
• Mild nausea/diarrhea managed at home • Fatigue affecting daily activities • Taste changes
• Mild mouth sores • Skin dryness or mild rash • Hair thinning • Nail changes
• Mild cold sensitivity • Mild tingling • Sleep problems • Mood changes
• Weight changes >5 lbs/week • Mild ankle swelling • General weakness

TREATMENT-SPECIFIC SYMPTOM ALERTS

Watch for these symptoms based on your specific treatment:

Treatment	Watch Especially For
FOLFOX	Cold sensitivity, neuropathy (numbness/tingling), throat tightness with cold, neutropenic fever
FOLFIRI	Severe diarrhea (can be sudden), neutropenic fever, nausea/vomiting
Capecitabine (Xeloda)	Hand-foot syndrome, diarrhea, chest pain (cardiac toxicity), mouth sores
Bevacizumab (Avastin)	High blood pressure, bleeding (any site), blood clots (DVT/PE), slow wound healing, GI perforation
Cetuximab / Panitumumab	Acne-like rash, severe infusion reactions, low magnesium (muscle cramps, weakness)
Pembrolizumab / Nivolumab	Immune-related: colitis (diarrhea), pneumonitis (cough/SOB), hepatitis, thyroid issues, skin reactions, any new organ symptoms
Regorafenib (Stivarga)	Hand-foot syndrome, fatigue, high blood pressure, liver toxicity (jaundice), bleeding
Lonsurf (TAS-102)	Severe neutropenia (fever risk), fatigue, nausea, anemia symptoms

YOUR IMPORTANT NUMBERS (Fill In)

Oncologist Office: _____	**After-Hours Line:** _____
Infusion Center: _____	**Nurse Navigator:** _____
Surgeon: _____	**Pharmacy:** _____
Nearest ER: _____	**Emergency Contact:** _____